Ebola virus disease (Bundibugyo), DRC: daily situation briefing

Generated 2026-10-19 08:16 UTC · national data to 2026-10-14 · zone data to 2026-10-11 · outbreak day 173 · ACTIVE · GRADE 3 · PHEIC since 2026-05-17

BDBV: No approved vaccine or therapeutic

Diagnostics: Standard Ebola RDTs do not detect BDBV

Cross-border: 20 Uganda · 1 France · **Kenya: 3 (1 imported (died), 2 household contacts detected in quarantine; no community transmission; MoH 10-16)** · Netherlands: 1 medevac HCW (under treatment)

Update 10-14 (INRB-UMIE): 9,277 DRC + 20 Uganda confirmed (+101), 4481 deaths (+54). DRC MoH (INSP): 924 suspected cases in isolation (10-14).

Situation

CONFIRMED CASES (DRC + UGANDA) - INSP MVE-128, 10-14

9,300

9,277 DRC · 20 + 1 prob. Uganda · 1 France · **2 Kenya** (+1 confirmed 10-16 by Kenya MoH, not yet in CDC total) · 1 Netherlands

CDC: DRC 10-14 · Uganda 10-14 · France 06-24 · INRB 10-14

CONFIRMED DEATHS (DRC + UGANDA) - INSP MVE-128, 10-14

4,481

4,478 DRC (48.3%) · 2 Uganda (10.0%)

Death proportion range 47-49% · Low confidence

SUSPECTED CASES IN ISOLATION - INSP, 10-14

924

Suspected cases in isolation, DRC MoH via INSP SitRep (2026-10-14). The national count of all suspected cases stopped being published after 4 Aug (270).

Each awaits a test result · Low confidence

CONTACT FOLLOW-UP — WHO DON619 (DATA 10-06)

29,535

23,741 monitored of 29,535 listed

Coverage 80.4% Below WHO 85% target

HEALTH ZONES WITH CONFIRMED CASES - INRB-UMIE MVE-125 (10-11)

65

Zones with >=1 confirmed case (INRB-UMIE zone-level data)

+20 Uganda confirmed + 1 probable (cross-border)

HIGHEST ATTACK RATE - INSP/INRB-UMIE, 05-28 (DRC-ONLY)

670.8

per 100,000 in Mongbalu (n=1,058) · Low confidence

Nizi: 631.5/100k

OUTBREAK PHASE (NOTIFICATION DATA, 7-DAY)

▲ Growth

+549 confirmed (+6%) over 8d (10-06→10-14). Notification-date only — may include diagnostic backlog.

Notification-date only. Linelist not available. Low confidence

This week national data to 14 Oct · zone data to 11 Oct 2026

549
New confirmed cases
▲ +28% vs previous 7 days (428)
national, 7 days to 14 Oct

43
Zones with new cases
of 65 zones ever affected

1
Newly affected zones
first case in the last 21 days

16
Zones quiet 21+ days
no new case for one incubation period

Where most new cases are (7 days to 11 Oct, zone data)

Watch list

HEALTH ZONE	PROVINCE	THIS WEEK	LAST WEEK	
Mandima	Ituri	**82**	56	▲
Beni	Nord-Kivu	**65**	56	▲
Bunia	Ituri	**53**	74	▼
Katwa	Nord-Kivu	**28**	41	▼
Nizi	Ituri	**27**	14	▲
Rwampara	Ituri	**18**	35	▼
Butembo	Nord-Kivu	**17**	23	▼
Mabalako	Nord-Kivu	**17**	15	▲

- **New: Alimbongo** (Nord-Kivu) — first case 2026-10-02, 5 confirmed so far
- **Back after a pause: Manguredjipa** — 2 new this week, none the week before
- **Back after a pause: Ganga** — 2 new this week, none the week before
- **Back after a pause: Aru** — 1 new this week, none the week before
- **Back after a pause: Logo** — 1 new this week, none the week before
- **Back after a pause: Adja** — 1 new this week, none the week before
- **Reported, unconfirmed: Nord-Ubangi** — 2 cases reported 2026-10-15 (Media citing officials (TimesLive/Reuters, 2026-10-15))
- **Kenya: local household transmission** — 3 cases (1 imported, 2 household contacts), last confirmed 2026-10-16
- **Quiet 21+ days (16):** Kambala, Goma, Miti Murhesa, Rumba, Lubunga (Tshopo), Ariwara, Mahagi, Wanierukula …

Source: INSP SitRep via INRB-UMIE, zone-level cumulative confirmed cases (latest zone data 2026-10-11; zone data usually lags the national total by a few days). “Quiet” means no new confirmed case was *reported* — zones cut off by insecurity (e.g. Fataki, Drodro, Lita) can look quiet because samples and reports are not getting through. 21 days = one maximum incubation period.

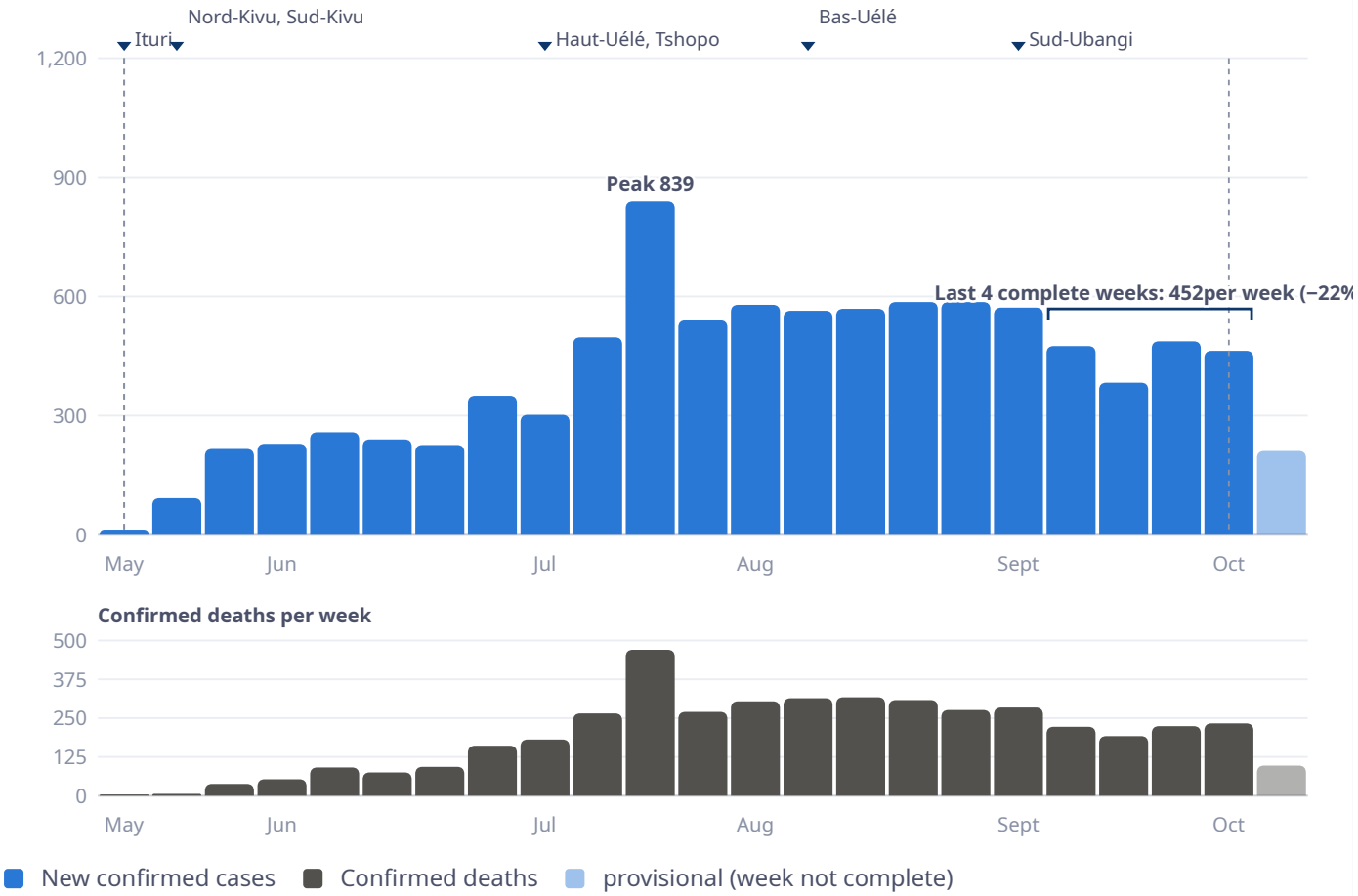
Key messages

- **Cases are falling slowly.** 452 confirmed cases per week over the last 4 complete weeks (14 Sept–11 Oct), −22% compared with the 4 weeks before. The peak was the week of 20 Jul 2026 (839 cases). Single weeks swing with reporting batches, so compare 4-week averages rather than one week.
- **The outbreak is shifting toward North Kivu.** It accounted for 35% of zone-attributed cases in the last 4 weeks, up from 18% before. 49 health zones reported cases in the last 3 weeks.
- **Mortality remains very high.** 218 confirmed deaths per week over the same period; deaths reported equal 48% of cases reported (a crude ratio, not a case fatality rate).
- **Transmission: Rt around 1.00** (95% CI 0.91–1.09) on 2 Oct 2026, the latest estimate not distorted by reporting delays: the outbreak is neither clearly growing nor clearly shrinking.

Computed automatically from INSP series via INRB-UMIE · data to 14 Oct 2026

Time

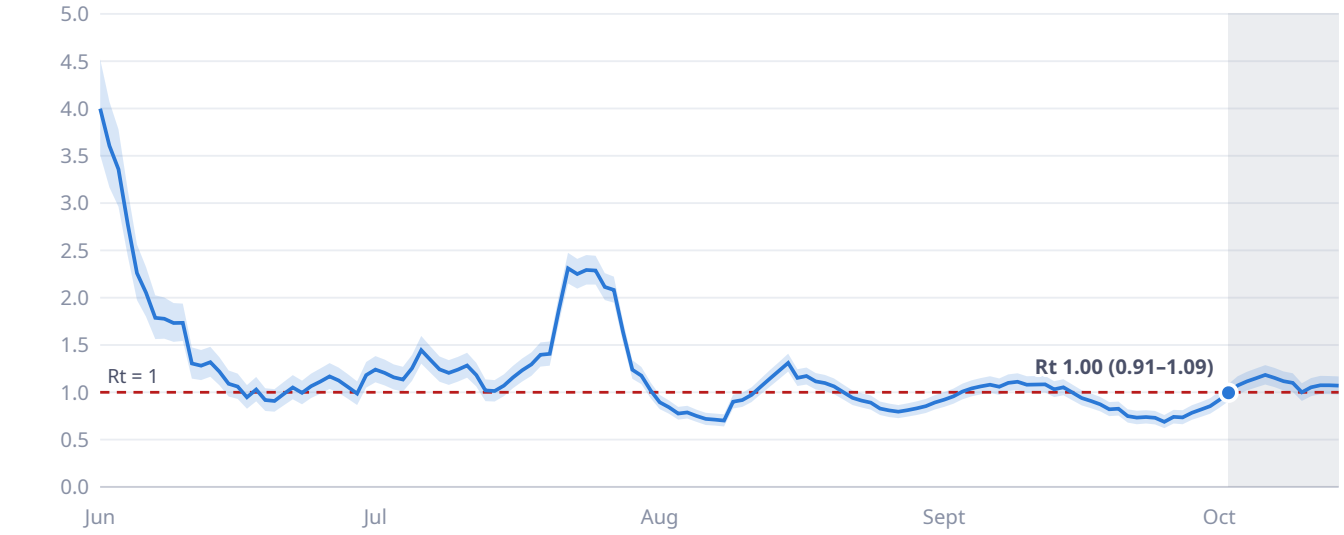
New confirmed cases per week



Source: INSP SitRep via INRB-UMIE · data to 14 Oct 2026

Confirmed cases by date of notification, not date of illness onset; recent days fill in as reports arrive. Weekly view is the default because daily counts move with reporting batches. Dashed lines mark the outbreak declaration, the PHEIC and the Kenya importation; triangles at the top mark the first week each province reported cases; the bracket shows the average of the last 4 complete weeks and its change. The panel below shows confirmed deaths per week on the same time axis. Hover any bar for exact values.

Is the outbreak growing or shrinking? Reproduction number (Rt)



Rt 95% interval Rt = 1: each case infects one other person on average

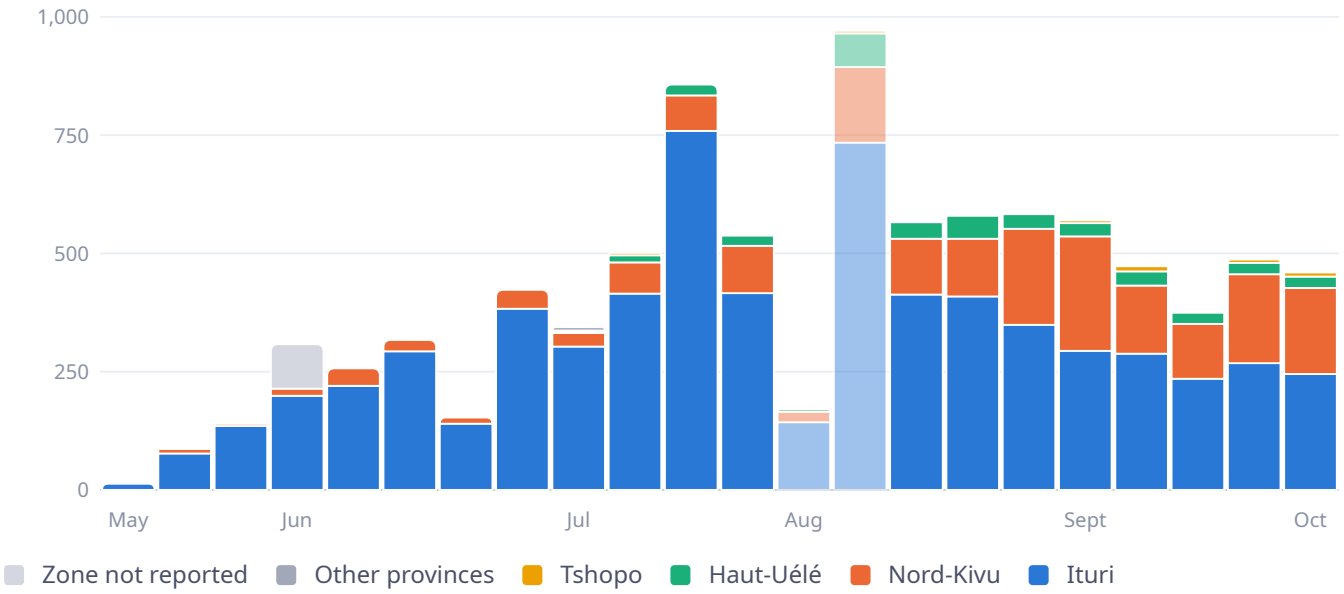
Last 12 days: likely underestimated (reporting delays)

Source: Cori et al. 2013 (EpiEstim sliding-window posterior), serial interval 11.4 d, on INSP national series via INRB-UMIE · data to 14 Oct 2026

Rt is the average number of people each case infects. Above 1 the outbreak grows, below 1 it shrinks. The shaded band is the 95% interval: when it includes 1, we cannot say which way the outbreak is heading. The last 12 days (grey) are biased low because recent cases are still being reported. Estimated from notification dates, not onset dates, so sharp spikes (for example in late July, the week of the 839-case peak) can reflect batches of delayed reports rather than real changes in transmission.

Place

New confirmed cases per week, by province



Source: INSP SitRep via INRB-UMIE · data to 11 Oct 2026

Built from health-zone data, which usually lags the national total by a few days. Lighter bars mark weeks where zone reports were incomplete or catching up (the early-August dip and jump is a reporting artefact; the national chart above is smooth). “Other provinces” groups Sud-Kivu, Bas-Uélé, Sud-Ubangi and zones not in the national reference; “Zone not reported” are cases reported without a health zone. “Show data revisions” shades weeks in which zone counts were revised downward.

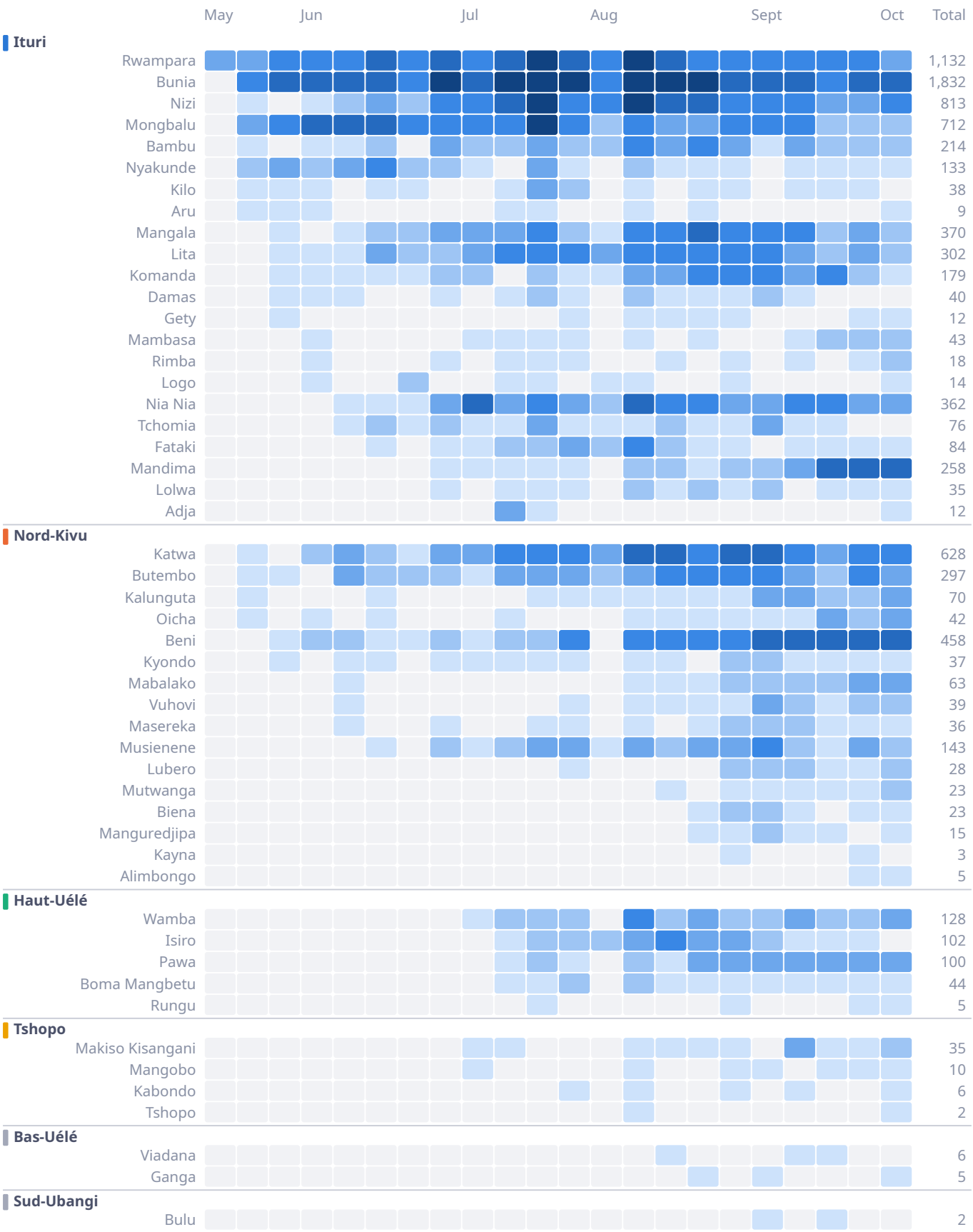
Affected provinces and health zones

PROVINCE	CONFIRMED	ZONES AFFECTED	ACTIVE (21 D)	MOST RECENTLY AFFECTED ZONE
Ituri	6,731	28/36	21	Gety (first case 30 Jul 2026)
Nord-Kivu	1,911	17/34	16	Alimbongo (first case 2 Oct 2026)
Haut-Uélé	383	7/13	5	Dungu (first case 18 Sept 2026)
Tshopo	63	7/23	4	Tshopo (first case 15 Aug 2026)
Bas-Uélé	12	3/11	2	Ganga (first case 24 Aug 2026)
Sud-Kivu	6	1/34	0	Miti Murhesa (first case 11 Jul 2026)
Province not identified	4	1	0	Rumba (first case 11 Jul 2026)
Sud-Ubangi	2	1/16	1	Bulu (first case 10 Sept 2026)

Source: INSP SitRep via INRB-UMIE zone data, data to 11 Oct 2026; zones per province: Africa CDC Sitrep #132, October 2 2026

Active = at least one confirmed case reported in the last 21 days. Zone-level totals come from the INSP zone series and can differ from WHO province totals, which use a different reporting cut-off. Uganda's outbreak ended on 28 July 2026; Kenya and other countries are on the Situation tab.

Where transmission is happening: health zones by week



New confirmed cases per week: 0 1–4 5–9 10–19 20–49 50–99 100+

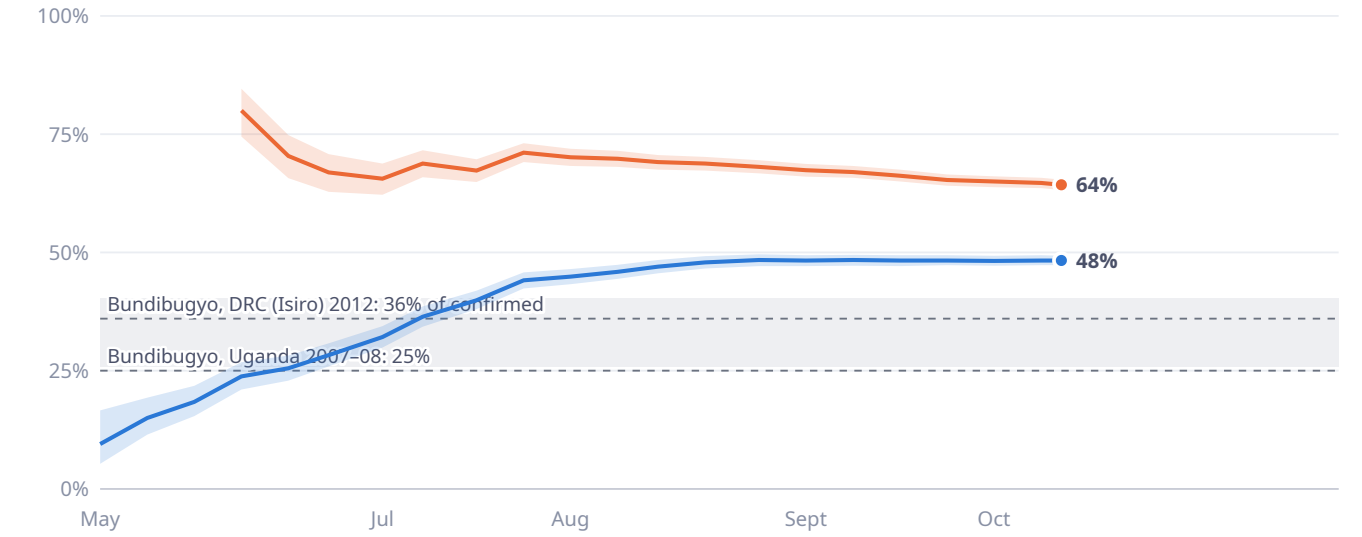
Source: INSP SitRep via INRB-UMIE · data to 11 Oct 2026

Each row is a health zone, grouped by province (Ituri, then Nord-Kivu, Haut-Uélé, Tshopo, others) and ordered by first case; each column is a week. Darker = more new confirmed cases. Read across a row to see when a zone was affected and whether it is still

reporting cases; a run of grey at the right end means no reported cases recently — in insecure zones this can also mean reports are not getting through. Shown: zones with at least 20 cases or any case in the last 3 weeks.

Severity and laboratory

How deadly is this outbreak? Deaths among confirmed and resolved cases



Source: INSP SitRep via INRB-UMIE · data to 14 Oct 2026

Reading the two lines: deaths divided by all confirmed cases (blue) is a lower bound, because many patients are still ill; deaths divided by cases with a known outcome (orange) can overstate the true fatality if recoveries are reported less completely than deaths. The true case fatality ratio most likely lies between them. **Past Bundibugyo outbreaks:** Uganda 2007–08: 25% (37 deaths among 149 cases); DRC (Isiro) 2012: 36% among 36 confirmed cases (51% including probable cases); pooled 33% (95% CI 26–40%). This outbreak is markedly deadlier, which may reflect late presentation, overwhelmed treatment capacity and community deaths as well as the virus.

Moderate confidence

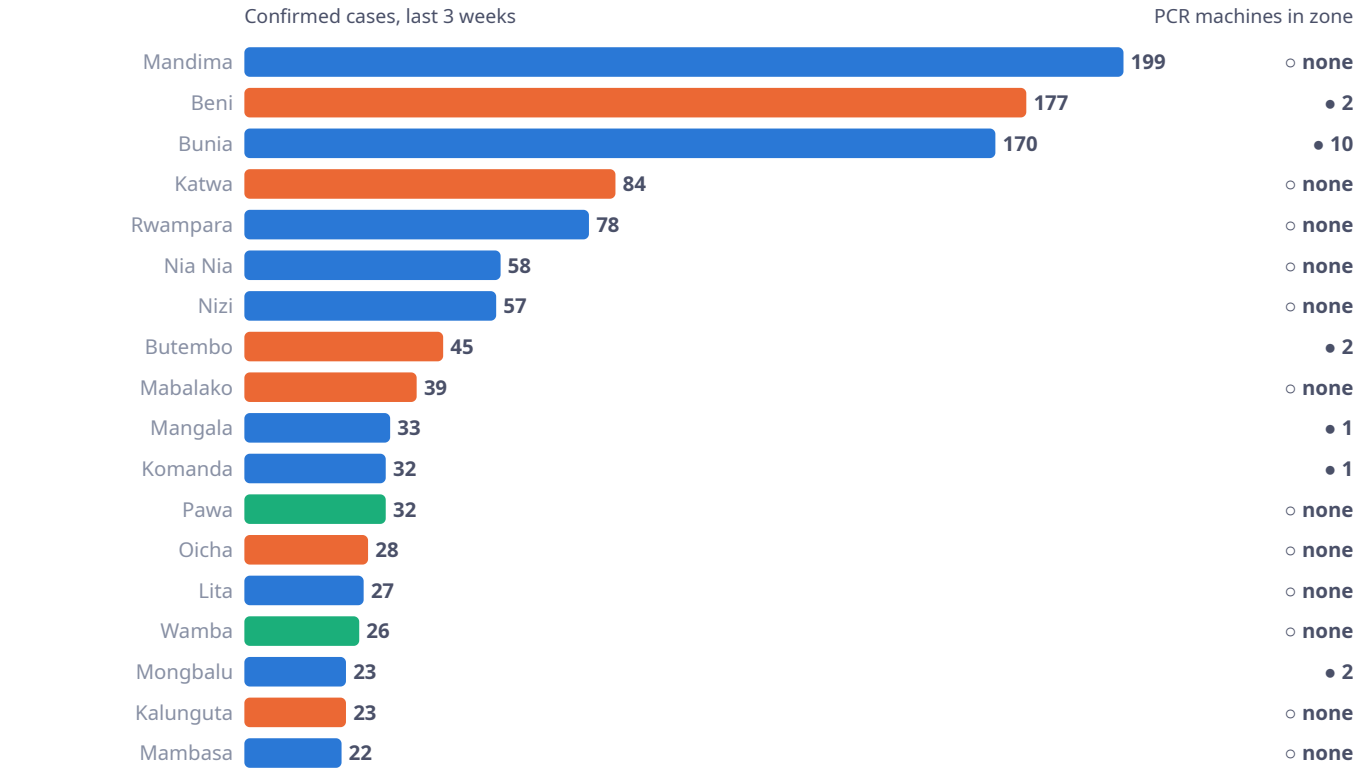
Crude death proportion: --% (right-censored lower bound — active cases unresolved)

Deaths among resolved cases: 64.3% (95% CI 63–65%) Method: deaths/(deaths+recovered)

Deaths divided by cases with a known outcome (6,964 resolved: died or recovered; INSP DRC series). Can overstate the true fatality if recoveries are reported less completely than deaths; the crude 48.3% is a lower bound. Past Bundibugyo outbreaks: 25% (Uganda 2007), 36% of confirmed (DRC 2012), pooled 33%.

Testing capacity versus where cases are now

38 of 49 health zones that reported cases in the last 3 weeks have no PCR machine listed; they account for **835 of 1,332** recent cases (63%).



Source: INSP SitRep via INRB-UMIE (cases, data to 11 Oct 2026); INRB-UMIE testing-capacity list (36 machines; listing date not stated)

Health zones with the most confirmed cases in the last 3 weeks, and the number of PCR machines listed in each. In zones without a machine, samples must be transported to another zone's laboratory, which adds days between sampling and result — and therefore between a case and the isolation and tracing of contacts. Machine counts show capacity, not throughput; staffing, reagents and security also limit testing.

Contacts and response

Are contacts being followed up?

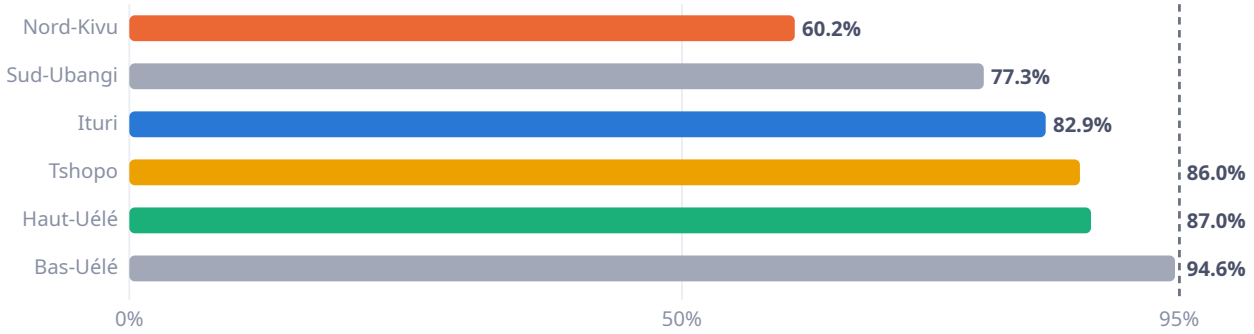
- **80.4% of contacts followed up** according to WHO (23,741 of 29,535, data of 6 Oct 2026), below the 85% benchmark. About **5,794 contacts** were not seen.
- **Nord-Kivu is the weak link:** 60.2% daily follow-up on 2 Oct 2026, so about 4 in 10 contacts were not seen that day. 6 of 6 provinces are below the 95% target.
- **No continuous series:** only occasional snapshots are published, and definitions differ by source. They do not form a trend.
- **Missing indicator:** the share of new confirmed cases who were already on a contact list is not published. That is the figure that shows whether tracing is catching transmission chains.



Source: Africa CDC Sitrep #24 · Africa CDC Sitrep #85 · WHO DON618 (publication date) · Africa CDC Sitrep #132 · WHO DON619

Each point is a published figure, not a daily series. Africa CDC counts contacts seen that day out of contacts due that day (or under follow-up out of listed); WHO counts contacts monitored out of contacts requiring follow-up. The dotted lines only link figures from the same source; do not read them as a trend. The grey band marks the period with no newer published figure.

Daily follow-up by province

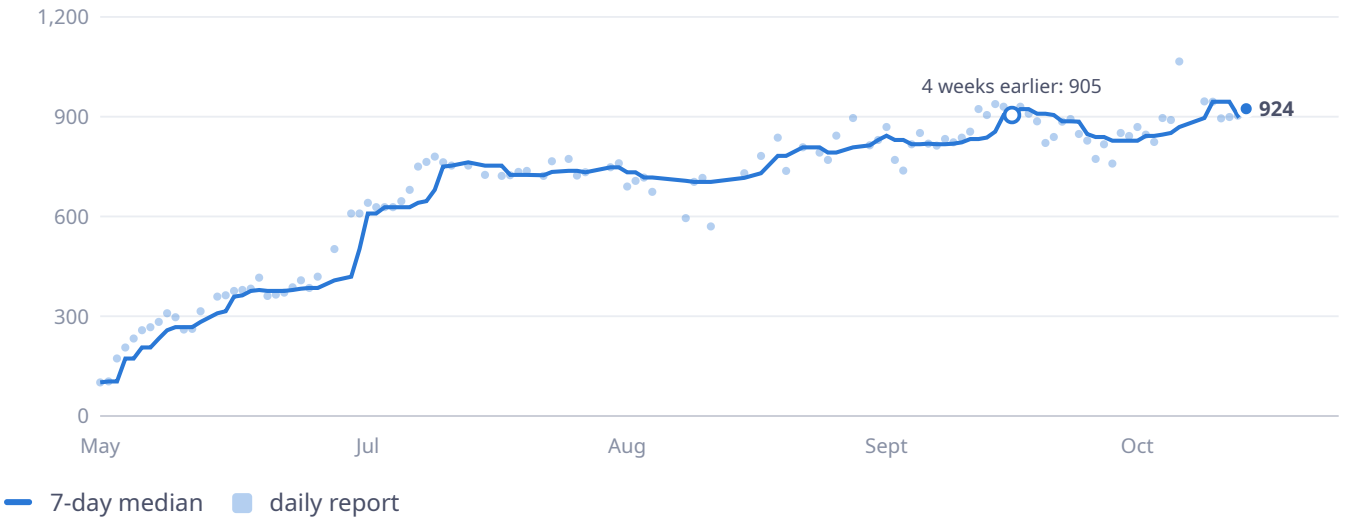


Source: Africa CDC Sitrep #132, October 2 2026 · Africa CDC daily target 95%

Share of contacts due for a visit who were seen that day, latest Africa CDC sitrep with a province breakdown. A contact who is missed for several days can develop symptoms and transmit before being found.

Is the isolation load easing?

- **924 suspected cases in isolation** on 14 Oct 2026, against 905 four weeks earlier (+2%, 7-day median), while confirmed cases fell 22%.
- Every isolated person needs a bed and staff until their test result is back. Isolation capacity is therefore still needed at the same level, even as confirmed cases fall.



Source: INSP SitRep via INRB-UMIE · data to 14 Oct 2026

National number of suspected cases held in isolation on each report day (INSP SitRep), not a cumulative count. Single-day jumps can be reporting artefacts, so the line shows the 7-day median.

Response gaps by province

Where the response is falling behind: contact follow-up below the 95% target, high bed occupancy, high death proportion.

PROVINCE	NEW CASES 7 DAYS TO 10-11	CUMULATIVE	DIED (OF CONFIRMED)	CONTACTS SEEN DAILY, TARGET 95%	ETU BEDS OCCUPIED	HEALTH ZONES AFFECTED/TOTAL
Ituri	243 ▼	6,370	46.0%	**82.9%**	37.2%	28/36
Nord-Kivu	182 ▼	1,657	**59.1%**	**60.2%**	63.0%	17/34
Haut-Uélé	24	352	41.5%	**87.0%**	53.6%	7/13
Tshopo	9 ▲	48	33.3%	**86.0%**	45.2%	7/23
Bas-Uélé	2 ▲	10	40.0%	**94.6%**	–	3/11
Sud-Kivu	0	3	33.3%	–	–	1/34
Sud-Ubangi	0	2	50.0%	**77.3%**	–	1/16

New cases: INSP via INRB-UMIE zone data (2026-10-11). Cumulative, deaths, contacts, beds and zones: Africa CDC Sitrep #132, October 2 2026 report 17 days old. Africa CDC publishes irregularly; newer national totals are on the Situation tab. Red follow-up =

below 80%, amber = below target.

Humanitarian impact

What the outbreak costs people beyond Ebola cases: displacement, hunger, children left without carers, disrupted routine care, and unfunded response.

People

1.1 million+
People displaced in Ituri
incl. 373,000+ living in displacement sites
UN News / OCHA, 2026-09-28

4.4 million
Displaced people in the five provinces with confirmed cases
Ituri, North Kivu, South Kivu, Haut-Uélé and Tshopo
UN News / UNHCR, 2026-08-06 74 days old

1.7 million
People in Ituri facing crisis-level hunger or worse
more than a third of the province
WFP, 2026-10-06

1,900+
Children orphaned or separated by Ebola
1,581 in Ituri, 322 in North Kivu (UNICEF figures, likely an undercount)
UNICEF, 2026-09-28

Collateral damage

-42%
Drop in use of essential health services in hotspots
Bunia, Mongbwalu, Nizi, Rwampara, Apr-Jun 2026; -13% province-wide
UNICEF, 2026-08-05 75 days old

-69%
Drop in first-dose measles vaccination, Mongbwalu
23,000+ fewer routine child vaccine doses in May-June vs April
UNICEF, 2026-08-05 75 days old

108,643
Suspected measles cases nationally, Jan-Jul 2026
1,394 deaths; nearly half in North and South Kivu
MSF, 2026-07-19 92 days old

Funding

48%
DRC humanitarian response plan funded
of $2.13 billion, incl. $300 million for Ebola-related activities
OCHA, 2026-09-01 48 days old

9%
UNFPA Ebola appeal funded
of $17.1 million (June-November 2026)
UNFPA, 2026-10-01

Access and security incidents affecting the response

2026-10-01 · Ituri · UN News / OCHA
Insecurity restricts humanitarian access to Fataki, Drodro and Lita health zones; some aid organisations suspended activities.
Effect on the response: Sample transport Fataki-Bunia delayed; patient transfers disrupted

2026-09-30 · Ituri · UN News
Ebola transit centre burned down in Kigonze displacement camp (about 19,000 people) after soldiers entered the camp; thousands fled.

Effect on the response: Loss of isolation and triage capacity for the camp

2026-09-27 · Ituri · UN News / OCHA
More than 3,000 people fled Rhoe displacement camp (Djugu) after clashes intensified on 27 September.
Effect on the response: Population movement out of a monitored site

2026-09-15 · Nord-Kivu · UN News / OCHA
Fighting in Masisi displaced more than 24,000 people since mid-September.
Effect on the response: New displacement in an outbreak-affected province

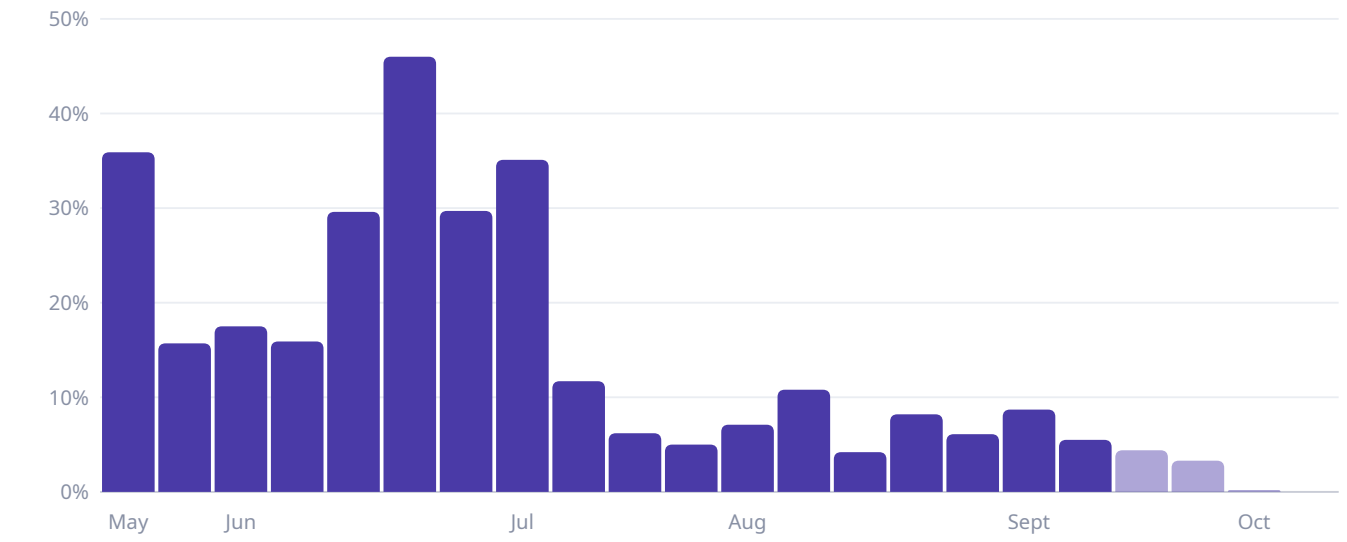
Curated from published UN, NGO and peer-reviewed reports; last reviewed 2026-10-18. Figures older than 45 days are greyed out. Incidents listed only as published by the cited source.

Genomics

Genomic surveillance: key facts

- **1,055 genomes** from the 2026 outbreak are public on Pathoplexus, about **11.3%** of confirmed cases (latest release 17 Oct 2026).
- **Sequencing has not followed the outbreak east:** 15% of Ituri cases have a genome, against 0.5% in North Kivu (10 genomes for 1,911 cases), where more than a third of cases now occur.
- **Coverage fell as the outbreak grew:** about 28% of cases were sequenced in June, against 7% over the last 8 complete weeks.
- **Delay:** a genome is released a median of 33 days after sample collection, so the most recent weeks are still incomplete.

Is sequencing keeping up? Share of each week's cases with a genome



Source: Pathoplexus (genomes by collection week) · INSP via INRB-UMIE (cases) · data to 17 Oct 2026

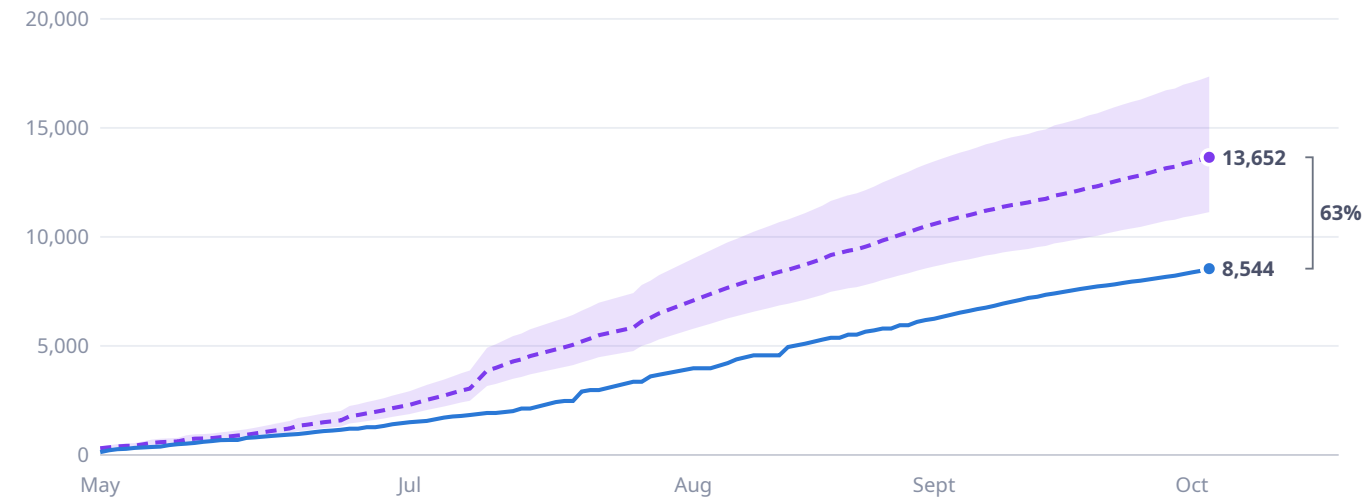
Genomes counted by week of sample collection, divided by confirmed cases notified that week. Faded bars are recent weeks that are still filling in, because genomes are typically made public about a month after the sample is taken. Most genomes are under Pathoplexus "restricted" terms: only counts are shown here.

Modelling

Key messages

- **Transmission: Rt 1.00 (0.91–1.09)** on 2 Oct 2026, not distinguishable from 1: neither clearly growing nor clearly shrinking. Later values are still incomplete. Weekly cases fell 22% over 4 weeks, which points the same way.
- **Unconfirmed cases:** if the true fatality ratio is close to earlier Bundibugyo outbreaks (33%), the 4,478 deaths imply about 13,652 cases, of which **63% (49–77%)** were confirmed. If the fatality ratio is higher in this outbreak, fewer cases are missing.
- **Next 4 weeks:** if Rt stays within its current interval, about 363 to 571 confirmed cases per week by early November (against 463 in the week of 5 Oct). A clear decline needs Rt to stay below 1.

How many cases may be unconfirmed?

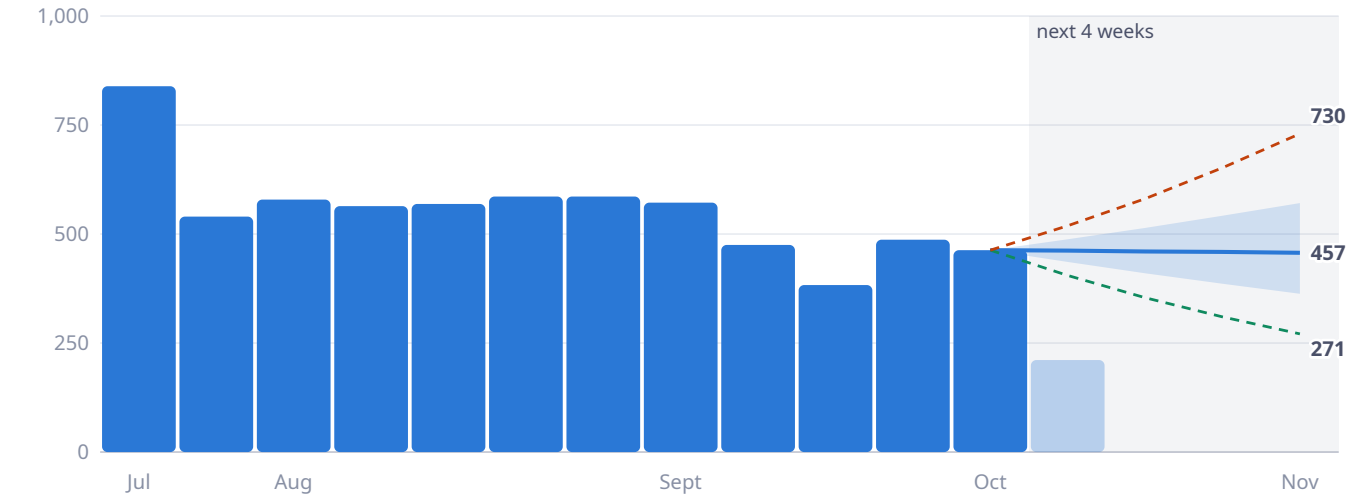


Confirmed cases — Cases implied by deaths (33%) — range for a fatality ratio of 26–40%

Source: INSP SitRep via INRB-UMIE · fatality ratio: Bundibugyo 2007 and 2012 pooled (PMC5124280) · data to 14 Oct 2026

Deaths are counted more completely than survivors. Dividing confirmed deaths by the fatality ratio seen in earlier Bundibugyo outbreaks gives the number of cases those deaths imply, dated 11 days earlier (mean time from onset to death). The result depends almost entirely on that ratio: in this outbreak 48% of confirmed cases and 64% of resolved cases died, which suggests either that mild cases are missed or that the disease is deadlier here. Both are likely partly true.

Where could the next 4 weeks go?



Confirmed per week — If Rt stays within its current interval (0.91–1.09) — Rt 0.8 (control) — Rt 1.2 (renewed growth)

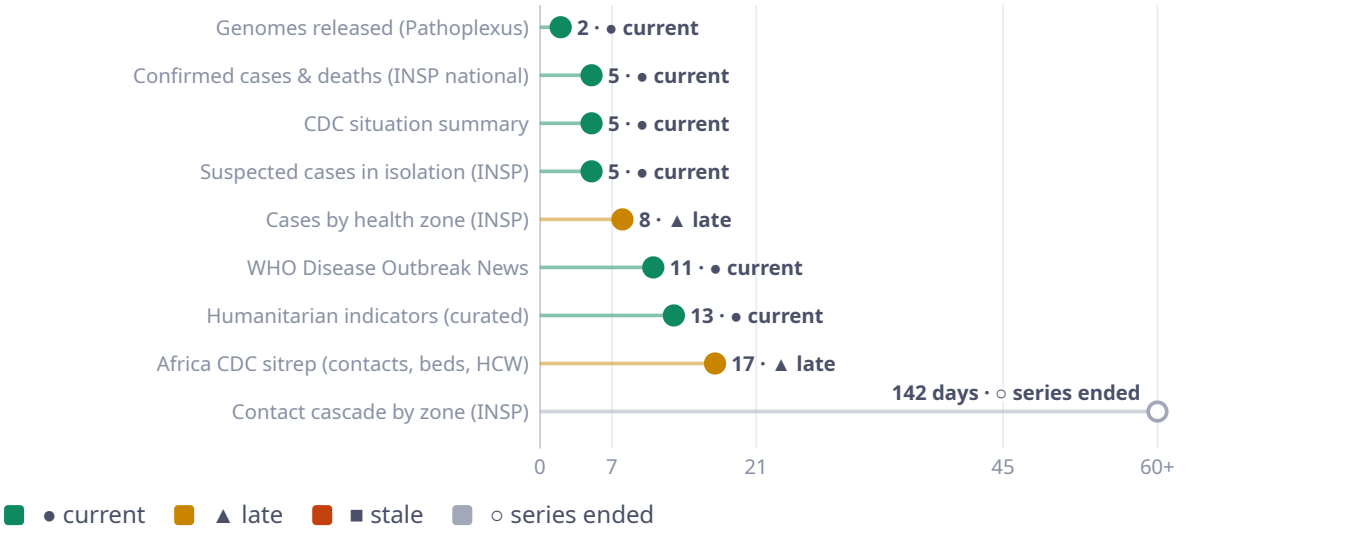
Source: INSP SitRep via INRB-UMIE · Rt: EpiEstim · Rt-to-growth conversion: Wallinga and Lipsitch, 2007 · data to 5 Oct 2026

Not a forecast. The band shows weekly confirmed cases if Rt stayed within its latest reliable interval; the dashed lines show what Rt 0.8 or 1.2 would mean. Projections of Ebola outbreaks lose accuracy quickly beyond 2 to 4 weeks, so the horizon stops at 4 weeks. Changes in testing, access or reporting move the observed counts independently of transmission.

Data quality

How current is each number?

- **Behind schedule:** Cases by health zone (INSP) (11 Oct 2026); Africa CDC sitrep (contacts, beds, HCW) (2 Oct 2026). Figures that depend on it (contact follow-up, beds, health workers) are older than the case counts.



Days between today and the latest data date of each source (not the date we checked it). Status compares that age with how often the source normally publishes: current = within a few days of its usual rhythm, late = overdue, stale = more than three weeks or twice the usual interval. A late source can mean the publisher paused, not that our pipeline failed; each source is checked automatically.

NOT PUBLISHED AT ALL

- **Case-level data** (age, sex, onset date, outcome): no public linelist, so the epidemic curve uses notification dates.
- **Share of new cases already on a contact list:** the key test of whether tracing is catching transmission chains.
- **Testing cascade** (samples collected, tested, positive, turnaround): only occasional backlog figures.
- **Location below health-zone level** (health area, village).
- **Genomes linked to cases:** most 2026 genomes are under restricted-use terms and cannot be linked to case data publicly.

Independent secondary-surveillance briefing generated by Atlas Outbreak Intelligence from public sources (INSP SitRep via INRB-UMIE, WHO, CDC, Africa CDC, Pathoplexus and cited reports). Not an official report of WHO, the DRC Ministry of Health, MSF or any agency. Figures are provisional and use notification dates. Interactive version and sources: outbreak.loew-beer.at. Contact: office@loew-beer.at. Analysis licence CC-BY-4.0; third-party data keeps its licence.